

Rib Fracture Care Plan

For LRI wards

Date DD/MM/YY

Time HH:MM
use 24h clock

Patient details

Full name

DoB

Unit number

(use sticker if available)

Patient's team to ensure that each section on this form is completed by the relevant clinician

1

Deliver humidified oxygen

- Maintain SpO₂ at 94-98% (88-92% if raised CO₂ on BG)
- Use a controlled O₂ device if raised CO₂ on BG
- Oxygen at flow rates ≥4L/min should be humidified

Time completed

Ward doctor print name and signature

Reason not done

☐ SpO₂ in room air within range

2

Prescribe effective analgesia

- Regular paracetamol
 - Regular NSAID unless contraindicated
 - Regular Dihydrocodeine (in the elderly: codeine)
 - If pain severe, consider regular oxycodone
 - PRN IV / PO morphine for break-through pain
- NB:** In patients with an eGFR <30 there is a risk of opiate toxicity due to accumulation; use tramadol instead of codeine / dihydrocodeine and oxycodone instead of morphine

Time drug chart completed

Ward doctor print name and signature

Reason not done

3

Pain team review

Daily review by the Adult Pain Management Team Mon-Fri to optimise pain control; requires ICE referral

Time of first review

Pain team print name and signature

Reason not done

☐ Pain well controlled

4

Physiotherapy

- 1st review within 24h of admission; further input guided by assessment as per Physiotherapy Rib Fracture SOP
- Aim is to maintain lung volume, prevent and treat lung collapse and consolidation, aid secretion clearance and facilitate mobilization
- Eligible pts will have received incentive spirometer in ED

Time of first review

Physiotherapist print name and signature

Reason not done

5

DART review

- 1st review within 14h of admission
- Criteria for escalation to ITU care (unless ceiling of care below ITU previously agreed) include
 - SpO₂ below target despite high flow oxygen therapy
 - Prolonged need for high flow (≥60%) oxygen therapy
 - Acute hypercapnia
 - Inability to speak in sentences

Time of first review

Outreach team print name and signature

Reason not done

6

Dietician review

Within the next working day; refer via ICE

Time of first review

Dietician print name and signature

Reason not done

ALL patients in whom severe pain remains an issue after 24h despite optimal systemic analgesia should be discussed with the thoracic surgical team for consideration of catheter-based regional analgesia / PCA.

Where appropriate, patients will be accepted for transfer to a thoracic surgical bed at the GGH. Please involve the LRI Senior Manager On Call (SMOC) if transfers are delayed. **NB:** Any delay in excess of 48h should be datixed.